

Rental review and flex

What the rates should be, and how far they can move.

01 / TWO LOOSE ENDS

Consequences of recent decisions, not yet carried through.

Medlyfe has no home. Medbury Diagnostics took the guest chalet, which was Medlyfe's. The laboratory alone fills all 80 sqm, so there is nothing left in it. Medlyfe's NGN 36.5M lease has quietly left the model and nobody decided that it should.

The obvious fix is worse than the problem. Three first-floor rooms at NGN 456,000 per sqm would yield about NGN 26M, but those same rooms sold sessionally earn about NGN 88M. **Leasing to Medlyfe would destroy NGN 62M of value.** Recommend Medlyfe does not take space in phase one and buys sessions like every other user, which is the same conclusion already reached for The Cloister.

Break-even has moved a long way. With Medlyfe gone and diagnostics and pharmacy treated as facility rather than tenants, only two occupiers pay anything. Fixed lease income is now the Conversion Clinic alone at NGN 68M against a fixed cost base of NGN 185.5M. Break-even moves from the 6% of stabilised plaza utilisation quoted earlier to about **26%**. Still safe, but a materially different risk profile, and the earlier headline should not be repeated.

02 / WHAT PAYS

Two occupiers, and the cost each has to carry.

Occupier	sqm	share	Allocated cost	Revenue	Margin
The Conversion Clinic	132	38%	70.2	68.0	(2.2)
Plaza, 9 consulting rooms	169	48%	89.8	430.3	340.5
Clean procedure room	28	8%	14.9	44.2	29.3
Treatment room	20	6%	10.6	29.8	19.2
	349		185.5	572.3	386.8

The Conversion Clinic at NGN 68M sits below its allocated cost of NGN 70M. That figure was set in the SPV waterfall as a plausible-looking number and never tested against what the space costs to provide. It is underpriced, not overpriced, and the fit-out is why.

Why the fit-out changes the benchmark

The standard lease terms in the original Medbury deck say **fit-out is tenant funded**. Here The Lyfe Place funds it. So a conventional per-sqm comparable is the wrong benchmark entirely, because the comparable tenant paid for its own. The rent has to carry that capital and earn a return on it.

Capital deployed	NGN M	Basis
Share of PropCo fit-out	61.3	Ground floor is 40% of the building, the Conversion Clinic 66% of the ground floor
Share of the 25KVA power system	12.5	By let-bearing area
Share of rent prepay, agency, legal and caution	43.5	By let-bearing area
Capital attributable to its space	117.3	Deployed by The Lyfe Place, not by the SPV

Three derivations, independently

Method	Working	NGN M
A Allocated operating cost plus a return on capital	NGN 70.2M + 20% on NGN 117.3M	93.7
B Bare lease plus fit-out annuity plus the serviced element	NGN 56.1M at NGN 425k/sqm + NGN 14.6M fit-out over 10 years at 20% + NGN 22M for reception, nursing, credentialing and imaging access	92.7

C	What the SPV waterfall carries at a 15% conversion fee	From the structuring note	95 to 112
---	--	---------------------------	-----------

Recommend NGN 95M	NGN 719,700 per sqm, or 1.7x a conventional medical suite, which the serviced element and the landlord-funded fit-out both justify	95.0
--------------------------	---	-------------

But rent and conversion fee are coupled, and the rent cannot be raised in isolation. At NGN 95M the clinic needs at least a **13% conversion fee** to leave anything behind to sustain itself. At a 10% fee the residual is negative by NGN 6M and the clinic cannot carry its own rent. Negotiate the two together or neither number means anything.

03 / BAND PRICING

Inverted against the only benchmark we have.

The Ikoyi work priced a six-hour consulting session at NGN 109,000, which is NGN 18,167 an hour. That is the only comparable in the house and Abuja should not sit far from it.

Band	Rate	Hrs	NGN/hr	vs Ikoyi	Parity	NGN M/yr
Early, 07:00 to 09:00	44,000	2	22,000	+21%	36,300	38.3
Daytime, 09:00 to 13:00	61,000	4	15,250	-16%	72,700	60.6
Afternoon, 13:00 to 17:00	61,000	4	15,250	-16%	72,700	53.0
Evening, 17:00 to 21:00	94,000	4	23,500	+29%	72,700	128.4

Evening is 29% above Lagos per hour and carries 46% of session revenue. It is simultaneously the least defensible rate and the one the model leans on hardest, which is the wrong combination. Daytime and afternoon sit 16% below Lagos, and that is correct rather than an error: they are the weak bands at 35% to 40% fill and they need to be cheap. Do not raise them to close the gap, because price is not what is holding them back.

04 / FLEX, LINE BY LINE

Floor, target, ceiling.

Line	Floor	Target	Ceiling	Flex, and what it is worth
Conversion Clinic licence	NGN 70M	NGN 68M	NGN 112M	None down. NGN 44M up. Below its own cost, and the SPV waterfall carries NGN 95M to 112M at a 15% conversion fee
Evening block	NGN 40k	NGN 94k	NGN 73k	NGN 21k down. Target already exceeds the defensible ceiling. Full concession costs NGN 28.7M a year
Early block	NGN 19k	NGN 44k	NGN 36k	NGN 8k down. Also above Lagos per hour. Full concession costs NGN 7.0M
Daytime and afternoon	NGN 26k	NGN 61k	NGN 73k	NGN 12k up on paper, but do not take it. These bands are demand-constrained, not price-constrained
Clean procedure session	NGN 108k	NGN 480k	NGN 550k	NGN 70k up. Cost-plus with the anaesthetist passed through. Worth NGN 9.7M
Treatment room session	NGN 64k	NGN 180k	NGN 220k	NGN 40k up. Worth NGN 6.6M
Membership, per member	n/a	NGN 2.5M	unknown	Untested in either direction. 25% of campus revenue resting on no benchmark at all

The net position is comfortable. There is about **NGN 43M of upward room** in the underpriced lines, against about **NGN 36M of downward room** likely to be conceded on the overpriced ones. The raises more than cover the concessions, so the campus can absorb a full negotiation on session rates without moving break-even, provided the Conversion Clinic licence is corrected at the same time.

05 / CONCENTRATION

Where the revenue actually sits.

Line	NGN M	Share	Exposure
All four session bands	280.3	47%	Fill assumptions borrowed from Ikoyi, unvalidated for Abuja
Memberships	150.0	25%	60 members at NGN 2.5M. No benchmark, no pilot, no signed member

Evening band alone	128.4	21%	The single most exposed line: highest rate, thinnest justification
Clean procedure room	71.8	12%	Depends on the anaesthetist roster being filled
The Conversion Clinic	68.0	11%	Contracted, but below cost and dependent on the conversion fee being agreed at all

Memberships and the evening band together are 46% of campus revenue, and both rest on the same unproven proposition: that Abuja's senior consultants will pay a premium to hold an evening list at a private address. If that is wrong, it is wrong for both lines at once. That correlation is the real risk in this model, not the break-even percentage.

06 / WHAT TO DO

Four moves, in order.

Move	Why
1 Raise the Conversion Clinic licence to NGN 95M, against a 13% floor on the conversion fee	From NGN 68M. Three independent derivations land between NGN 93M and 96M. The two numbers move together, so do not concede the fee and hold the rent
2 Hold the evening rate at NGN 94,000 but expect to concede	Open there, keep NGN 21k in reserve, and concede it against a longer membership commitment rather than for nothing
3 Decide Medlyfe explicitly	Recommend it buys sessions rather than takes space. Do not let a NGN 36.5M line disappear by accident
4 Pilot the membership before the model relies on it	Ten founding members signed at NGN 2.5M before fit-out completes would validate a quarter of campus revenue. Nothing else in the plan is cheaper to test

Debo Odulana Consult for Africa / +234 913 813 8553 / hello@consultforafrica.com

Cost allocation is by let-bearing sqm, a convention rather than the only defensible one. The Ikoyi rate is a Lagos figure and the only in-house comparable. Fill, membership pricing and conversion volumes remain unvalidated for Abuja. Not a binding offer. FX USD/NGN 1,550.